

54. All the following are indications for thoracentesis with the exception of;

- A. Nephritic syndrome
- B. Chylothorax
- C. Thrombocytopenia
- D. Pleurodosis

55. A nurse is administering nebulization therapy to a patient with asthma. The patient begins to report dizziness and tachycardia. What is the most appropriate nursing action?

- A. Continue the nebulizer treatment, as these are common side effects.
- B. Stop the nebulizer treatment and monitor vital signs.
- C. Ask the patient to lie down and rest for 30 minutes.
- D. Administer supplemental oxygen and continue the nebulization.

56. Nurse Sarah is attending to a patient with a tracheostomy and a high fever of 103° F (39.4° C). Which intervention is most likely to decrease the client's arterial blood oxygen saturation?

- A. Applying a cooling blanket
- B. Encouraging the patient to cough
- C. Performing endotracheal suctioning
- D. Using incentive spirometry

57. Nurse Lisa is caring for Mr. Anderson, who has a chest tube connected to a closed water-seal drainage system. What action should she include in his plan of care?

- A. Ensure there is continuous bubbling in the water-seal chamber.
- B. Measure and record the amount of drainage in the collection chamber
- C. Position the collection chamber at chest level
- D. Strip the chest tube every hour

58. A nurse is preparing a patient for a bronchoscopy to evaluate suspected lung cancer. The patient asks if they will be awake during the procedure. What is the most appropriate response?

- A. "You will be asleep during the entire procedure, so you won't feel anything."
- B. "You will be awake, but the procedure will be quick and you will be comfortable."
- C. "You will be awake but sedated, and local anesthesia will numb your throat to minimize discomfort."
- D. "You will be awake and conscious, but we will provide general anesthesia to make you comfortable."

59. A nurse is educating a patient about post-procedure care following a bronchoscopy. The nurse explains that the patient may experience mild symptoms afterward. Which of the following symptoms is considered normal and expected after a bronchoscopy?

- A. Severe chest pain and difficulty breathing.
- B. Persistent cough with blood-tinged sputum.
- C. Severe sore throat and hoarseness.

D. Sudden loss of consciousness or confusion

60. A nurse is caring for a patient post-laryngoscopy who is experiencing hoarseness and throat discomfort. Which of the following is the most appropriate intervention for the nurse to recommend?

- A. Use a humidifier and encourage voice rest.
- B. Encourage the patient to speak in a low, soft voice to prevent further irritation.
- C. Recommend the use of warm tea with honey every hour to soothe the throat.
- D. Advise the patient to avoid drinking fluids to reduce throat irritation.

61. After a barium meal, a patient reports that their stool has turned white or light-colored. What is the most appropriate nursing response?

- A. "This is normal and may cause your stool to appear white for a few days."
- B. "This is a concerning sign; it suggests a possible gastrointestinal obstruction."
- C. "This means you are not properly digesting food, and we need to schedule further tests."
- D. "You need to take a laxative immediately to relieve the stool discoloration."

62. A nurse is preparing a patient for a barium meal procedure. The patient asks what they should expect during the test. What is the most appropriate response?

- A. "You will be given a sedative to help you relax, and then a series of X-rays will be taken."
- B. "You will drink a chalky liquid, and then X-ray images will be taken to assess your digestive tract."
- C. "You will be given an injection of contrast material, and the doctor will use a scope to examine your stomach."
- D. "You will undergo a CT scan after swallowing a special contrast liquid."

63. Nurse Jones is preparing a patient for a paracentesis procedure to remove ascitic fluid. Which of the following is the most important pre-procedure action?

- A. Ensure that the patient has been NPO (nothing by mouth) for at least 12 hours.
- B. Check the patient's vital signs and assess for signs of infection.
- C. Ask the patient to empty their bladder before the procedure.
- D. Administer a sedative to ensure the patient remains relaxed during the procedure.

64. A patient is scheduled for a paracentesis procedure to remove ascitic fluid. The patient asks how the procedure is done. Which response is correct?

- A. "A small tube is inserted into your abdomen, and the fluid is drained into a bag."
- B. "A needle is inserted into your abdomen to remove the fluid, and you will be awake during the procedure."
- C. "An incision is made in your abdomen, and the fluid is drained through a drain for several days."
- D. "A catheter is inserted into your abdomen to remove the fluid, and you will be under general anesthesia."

65. After a paracentesis procedure, the nurse is monitoring the patient. Which of the following complications should the nurse be most alert for post-procedure?

- A. Infection at the puncture site
- B. Hemorrhage from the puncture site
- C. Cardiac arrhythmias due to fluid shifts
- D. Pulmonary embolism

66. To measure the required length on nasogastric tube, measure from the patient's:

- A. Nose to angle of the jaw to xiphisternum
- B. Ear to xiphisternum
- C. Nose to ear to xiphisternum
- D. nose to xiphisternum

67. A patient with an NG tube is receiving continuous feeding. The nurse notices that the patient is experiencing abdominal distension and discomfort. Which of the following actions should the nurse take first?

- A. Administer an antacid to relieve abdominal discomfort.
- B. Check for signs of tube displacement or obstruction.
- C. Discontinue the feeding and notify the physician immediately.
- D. Increase the feeding rate to help the patient tolerate the feedings better.

68. The most common side effect from successful nasogastric tube placement is.....

- A. aspiration pneumonia.
- B. a perforated abdomen.
- C. tissue trauma or irritation.
- D. vomiting.

69. The purpose of an NG tube is for feeding, prevention of nausea, vomiting, and gastric distention following surgery, to remove stomach contents, and to wash (lavage) the stomach.

- A. True
- B. False

70. An ileostomy is a surgical opening created to bring the large intestine to the surface of the abdomen.

- A. True
- B. False

71. A patient is scheduled for a liver biopsy. What is the most important pre-procedure instruction for the nurse to give the patient?

- A. "You will need to fast for at least 12 hours before the procedure."
- B. "You should take an anti-inflammatory medication to reduce pain during the procedure."
- C. "You will be required to stop any blood-thinning medications a few days before the procedure."
- D. "You will need to drink plenty of fluids the night before the procedure."

72. The following are all methods of taking sample for liver biopsy with the exception of:

- A. Percutaneous
- B. Transjugular
- C. Endoscopy
- D. Laparoscopy

73. A patient presents to the emergency department after ingesting a large quantity of an unknown toxic substance. The physician orders a gastric lavage. After the procedure, the patient becomes increasingly agitated and complains of shortness of breath. The nurse notices that the patient's oxygen saturation levels are dropping. What is the most appropriate first action by the nurse?

- A. Administer supplemental oxygen and monitor the patient closely for respiratory distress.
- B. Continue the gastric lavage to ensure complete removal of the substance.
- C. Reassure the patient that the agitation is normal after the procedure.
- D. Notify the physician and prepare the patient for an immediate chest X-ray

74. A patient is undergoing gastric lavage after ingesting a toxic substance. The nurse notices that the lavage returns are not clear and continue to contain the ingested substance. What is the most appropriate next step for the nurse?

- A. Continue the lavage until the returns are clear.
- B. Notify the physician that the patient may need activated charcoal.

- C. Reassess the patient's vital signs and prepare for possible complications.
- D. Stop the lavage immediately, as further lavage may lead to harm.

75. A nurse is assessing a patient who has a history of gastric lavage and multiple gastrointestinal surgeries, including the placement of a colostomy and ileostomy. The patient reports symptoms of nausea, bloating, and a change in bowel movement patterns. Which of the following is the most appropriate intervention?

- A. Administer an antiemetic and reassess the patient in 30 minutes.
- B. Assess the patency of both the colostomy and ileostomy to rule out obstructions.
- C. Encourage the patient to drink a large amount of water to relieve bloating.
- D. Continue to monitor the patient for 24 hours and provide reassurance.

76. Patients with an ileostomy are at greater risk for dehydration and an electrolyte imbalance.

- A. True
- B. False

77. A 50-year-old patient is undergoing a fractional test meal to evaluate gastric acid secretion. After the meal is administered, the nurse should expect to measure which of the following?

- A. Gastric pH levels
- B. Gastric acid volume
- C. Gastric bicarbonate secretion
- D. Gastric motility and emptying time

78. A nurse is explaining the fractional test meal procedure to a patient with a history of severe abdominal pain and bloating. The nurse should inform the patient that the meal administered during the procedure will:

- A. Be very small to ensure minimal discomfort.
- B. Contain high levels of fat to stimulate gastric acid production.
- C. Include a liquid substance to neutralize stomach acid.
- D. Be a standard meal designed to stimulate normal gastric function.

79. A patient is undergoing cystoscopy for evaluation of chronic urinary tract infections. The nurse should prepare the patient for which of the following potential complications of the procedure?

- A. Hemorrhage or significant blood loss
- B. Urinary retention or difficulty urinating
- C. Severe chest pain or respiratory distress
- D. Anaphylactic reaction to anesthesia

80. A patient is recovering from a ureteroscopy. The nurse should monitor for which of the following signs of a potential complication?

- A. Increased pain and swelling in the lower abdomen
- B. Decreased urine output, fever, and flank pain

- C. Sudden drop in blood pressure
- D. Severe back pain unrelated to urinary function

81. A nurse is caring for a patient after a needle biopsy of the kidney. Which of the following is a priority assessment post-procedure?

- A. Assessing the puncture site for signs of bleeding or hematoma formation
- B. Checking the patient's blood pressure every 15 minutes for the first hour
- C. Monitoring the patient's respiratory every hour for the next 2 hours
- D. Observing for signs of respiratory distress as a result of anesthesia

82. A patient who underwent a needle biopsy of the kidney reports feeling lightheaded and has a drop in blood pressure. The nurse should first:

A. Administer fluids and monitor the patient's vital signs closely.

- B. Assess for pain at the biopsy site and administer pain relief
- C. Notify the healthcare provider and prepare for a possible transfusion
- D. Place the patient in a supine position and elevate their legs

83. A patient is undergoing an IVP. The nurse observes that the patient is experiencing nausea and a sense of impending doom following the injection of the contrast dye. What should the nurse do next?

- A. Administer an antiemetic and continue monitoring the patient
- B. Check the patient's vital signs and prepare to intervene for an allergic reaction
- C. Reassure the patient that these sensations are normal and will subside
- D. Inform the healthcare provider and prepare for an allergic reaction

84. A patient undergoing hemodialysis has a newly placed arteriovenous (AV) fistula. The nurse should assess the fistula for which of the following?

- A. A pulse deficit in the upper extremity
- B. A bruit and a thrill over the fistula
- C. Swelling in the lower leg
- D. Coolness in the affected extremity

85. A patient on hemodialysis reports muscle cramps during the procedure. The nurse should first:

- A. Administer pain medication and continue with the dialysis treatment
- B. Discontinue the dialysis treatment immediately
- C. Increase the blood flow rate to improve circulation
- D. Slow the dialysis rate and assess the patient's blood pressure and electrolytes

86. A nurse is preparing to administer medications to a patient receiving peritoneal dialysis. What is an important consideration when administering medications?

- A. All medications should be withheld during dialysis due to the risk of interactions.
- B. Medications should be given during the exchange of dialysate to increase absorption.
- C. Medications should be administered through the dialysis catheter after the exchange.
- D. Oral medications are preferred as they are absorbed more effectively during dialysis.

87. A patient who has been on peritoneal dialysis for several months presents with a complaint of increased abdominal pain and cloudy dialysate. What is the nurse's most likely concern?

- A. The patient is experiencing dehydration due to fluid loss during dialysis.
- B. The patient has developed peritonitis, a possible infection of the peritoneum.
- C. The patient's catheter is blocked and needs to be replaced.
- D. The dialysis process is not removing enough fluid from the abdomen.

88. When performing otoscopy in an adult, pull their ear:

- A. downwards and backwards
- B. Horizontally
- C. upwards and backwards
- D. vertically

89. A relative contraindication to urethral catheterization is suspected urethral injury via blunt trauma. Which of the following is the most important sign that this type of injury has occurred in a male?

- A. Blood at the urethral meatus
- B. Inability to void
- C. Penile edema
- D. Perineal ecchymosis

90. When the balloon on an indwelling urinary catheter is blown up, the patient expresses discomfort. You should:

- A. Aspirate the fluid from the balloon and advance the catheter
- B. Continue to blow up the balloon, because discomfort is expected
- C. Pull back on the catheter slightly to determine tension
- D. Remove the catheter

91. A nurse is teaching a patient with a suprapubic catheter how to care for the catheter at home. Which of the following should the nurse include in the teaching?

- A. "Ensure the catheter remains tightly secured to avoid movement and irritation."
- B. "It is important to apply ointment to the insertion site to keep it moisturized."
- C. "You should irrigate the catheter with a sterile solution every day to prevent blockages."
- D. "You should change the catheter weekly to avoid infection."

92. A patient with a suprapubic catheter complains of pain and fullness in the lower abdomen. What is the nurse's most appropriate action?

- A. Check for kinks or obstructions in the catheter tubing.
- B. Continue monitoring and reassure the patient that this is normal.
- B. Flush the catheter with saline to relieve the fullness.
- D. Reposition the patient to improve drainage.

93. A nurse is caring for a post-operative patient who has a three-way Foley catheter in place after bladder surgery. Which of the following is the most important reason for using a three-way catheter in this patient?

- A. To ensure continuous drainage of urine.
- B. To allow for removal of clotted blood or debris.
- C. To monitor the patient's urine output.
- D. To prevent urinary tract infections.

94. A nurse is performing manual bladder irrigation on a patient with a three-way Foley catheter. The nurse notices that the irrigation fluid is not flowing freely into the bladder. What is the nurse's next best action?

- A. Change the catheter immediately to restore flow.
- B. Check the catheter and tubing for kinks or obstructions.
- C. Increase the flow rate of the irrigation fluid.
- D. Use a larger syringe to force the fluid into the catheter.

95. A patient with a short arm cast is complaining of increased pain and tightness in the cast. The nurse notices that the patient's fingers are swollen, and the cast feels very tight. What is the nurse's most appropriate action?

- A. Assess the neurovascular status of the fingers and notify the healthcare provider.
- B. Encourage the patient to wiggle their fingers to relieve discomfort.
- C. Reassure the patient that this is normal and will resolve in a few days.
- D. Loosen the cast to relieve pressure on the fingers.

96. A nurse is providing discharge instructions to a patient with a plaster cast on their leg. Which of the following instructions should be included in the teaching?

- A. "Elevate the leg regularly to reduce swelling and prevent pressure sores."
- B. "If the cast gets wet, you can use a hair dryer to dry it quickly."
- C. "You should apply heat to the cast to help it dry faster."
- D. "You can walk on the cast immediately after it's applied."

97. A patient is in traction for a hip fracture. The nurse is explaining the importance of proper body alignment. Which of the following statements by the patient indicates a need for further teaching?

- A. "I need to make sure my body is aligned properly to avoid further injury."
- B. "I should ask for assistance when I need to reposition in bed to avoid disrupting the traction."
- C. "I can move around in bed as much as I want, as long as I don't disturb the traction weights."
- D. "I should avoid twisting or turning my body while in traction."

98. A patient in traction is experiencing discomfort due to the traction device rubbing against their skin. What should the nurse do to prevent skin breakdown?

- A. Apply barrier cream to the affected skin area.
- B. Loosen the traction device to reduce pressure on the skin.
- C. Place a soft cloth or padding between the traction device and the skin.
- D. Remove the traction device and allow the skin to rest.

99. A nurse is caring for a patient with lower leg trauma in a splint. The patient is experiencing unrelieved pain, which worsens when the nurse palpates the leg. The nurse observes that the leg is tense and the skin appears shiny. What is the most likely cause of these symptoms?

- A. Deep vein thrombosis (DVT)
- B. Compartment syndrome
- C. Cellulitis
- D. Muscle strain

100. A newly diagnosed patient with Type 2 Diabetes Mellitus asks the nurse about the role of exercise in managing their condition. The nurse explains that regular exercise helps in which of the following ways?

- A. Increases insulin resistance and raises blood glucose levels.
- B. Helps in weight management and increases insulin sensitivity.
- C. Causes a decrease in insulin sensitivity, leading to higher blood glucose.
- D. Is not recommended for patients with diabetes due to the risk of hypoglycemia.

101. A nurse is caring for a diabetic patient who has peripheral neuropathy and poor circulation. Which of the following should the nurse recommend for nail care to help prevent infection or injury?

- A. "You should always have a professional pedicure every month."

- B. "Make sure to cut your toenails very short to avoid infection."
- C. "Use a soft file to smooth any rough edges instead of cutting your nails."
- D. "Use hot water to soak your feet before trimming your nails to make the process easier."

102. The following are all types of glucose test with the exception of:

- A. Fasting blood glucose test
- B. Random blood glucose test
- C. Skin blood glucose test
- D. Oral glucose tolerance test

103. Which of the following factors can influence a person's BMR?

- A. Age, gender, body composition, and temperature.
- B. Age, height and weight.
- C. Age, exercise level and food intake.
- D. Gender, height and exercise.

104. What is the most common symptom that may lead to the performance of a urethral smear?

- A. Unexplained weight loss.
- B. Dysuria / urethral discharge.
- C. Swelling of the lower abdomen.
- D. Foul-smelling urine.

105. A patient presents with purulent urethral discharge. The nurse is preparing the patient for a urethral smear. Which of the following is the correct technique for performing the procedure?

- A. Insert the swab directly into the bladder through the urethra.
- B. Insert the swab into the urethra without touching the surrounding skin.
- C. Collect a sample of urine and send it to the lab for testing.
- D. Flush the urethra with sterile saline before inserting the swab.

106. The instrument used for retraction of vaginal wall during inspection of the vagina and cervix is?

- A. Sim's Speculum
- B. Cusco's Speculum
- C. Tenaculum
- D. Doyen's retractor

107..... is a specially designed needle for putting stitch around the cervix.

- A. Sponge holding needle